

TRIAD Investor FAQ

Common Questions About the Investment Opportunity

July 2026

TRIAD: Frequently Asked Questions for Investors

About TRIAD & The Model

Q: Why is the integrated multi-department model better than three separate clinics?

A: Three key advantages:

1. **Patient convenience:** Families book once, get coordinated assessment across all specialties, single billing. This improves patient experience and retention.
2. **Operational efficiency:** Shared reception, IT infrastructure, admin, marketing. Shared services (hydrotherapy, wellness) leverage capacity across departments.
3. **Revenue synergies:** Cross-referrals between departments improve utilization. A parent with a stroke (neurology) often needs concurrent orthopedic recovery. A child with cerebral palsy benefits from both pediatric and orthopedic therapy.

Three separate clinics compete for the same patients; TRIAD captures them all.

Q: Why would patients choose TRIAD over established solo practitioners?

A: TRIAD offers five things solo clinics don't:

1. **Integrated care:** One appointment, all three specialties available.
2. **Professional environment:** Modern facility, IT systems, insurance panel integration.
3. **Founder expertise:** Each founder is a recognized specialist with 6+ years proven track record.

4. **Convenience:** Parking, reception, payment — all in one place.
5. **Trust:** Insurance company and corporate partnerships (employee wellness programs) often prefer established, multi-specialty centers.

For high-income New Cairo families, this is a premium positioning.

Q: Can the model scale to other locations?

A: Yes, absolutely. This is a **unit-economics model**: one location validates the business, then replication is capital-efficient.

By Year 3, a second location in similar demographics (e.g., Sheikh Zayed, Maadi) would: - Share marketing, HR, IT management (head office) - Use proven staffing/equipment templates - Reduce launch costs by 20–30% (vendor bulk discounts, known operations) - Break even faster (reputation pre-built)

Target: 3–5 locations by Year 7, positioning TRIAD as Egypt’s first multi-location rehabilitation group.

Financial Viability

Q: Why is therapy a good recurring-revenue business?

A: Therapy is among the stickiest B2C health services:

1. **Duration:** Patients attend 2–3x/week for 6–12 weeks (minimum), often longer.
2. **Outcomes:** Progress is visible; patients see improvement and continue.
3. **Referrals:** Therapist-patient relationships drive word-of-mouth and high referral rates.
4. **Insurance:** Private insurance covers ongoing therapy; reduces price sensitivity.

Churn is typically 5–15% monthly once patient finds “their therapist.” Contrast with a dermatology clinic (one-off visits) or surgery (one procedure).

Q: What are the key revenue drivers?

A: Three:

1. **Volume:** Clinical sessions (therapist-patient pairs). Each therapist does 6–8 sessions/day, 5 days/week. At 250 EGP/session (ortho), 300 EGP (neuro), 200 EGP (pediatric), a single therapist generates ~6M EGP annually.
 2. **Payer mix:** 40% out-of-pocket, 40% private insurance, 20% corporate wellness/employee programs. Insurance & corporate clients are higher-margin and lower-churn.
 3. **Shared services:** Hydrotherapy, wellness classes, home visits, corporate programs generate incremental revenue without significant additional therapist hiring.
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Q: What's the monthly cash burn until profitability?

A:

Budget Scenario	Start-up	Monthly Opex	Months to Breakeven
Essential	1.2M	180K	12–14
Recommended	1.8M	280K	10–12
Luxury	2.5M	380K	9–10

Key assumptions: 50% capacity utilization by Month 6; 70% by Month 9; 85%+ by Month 12. Based on conservative 3–4 months patient acquisition ramp.

Q: How much equity are you offering?

A: Negotiable based on investment amount and terms:

- **EGP 1.2M investment** → ~40% equity (Essential scenario)
- **EGP 1.8M investment** → ~45% equity (Recommended scenario)
- **EGP 2.5M investment** → ~50% equity (Luxury scenario)

Investors would receive board seat, quarterly financials, and a path to exit (dividend distributions or sale/expansion within 5–7 years).

Q: What's your exit strategy?

A: Two paths:

1. **Dividend distributions:** Once TRIAD is cash-positive (Month 12), distribute 30–50% of annual profits to investors annually.
 2. **Strategic sale or expansion:** Larger healthcare groups, hospital networks, or private equity may acquire TRIAD after 3–5 years of proven profitability. Estimated valuation at exit: 8–12x EBITDA (typical for successful healthcare operators in emerging markets).
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Risk & Mitigation

Q: What if patient acquisition is slower than projected?

A: Mitigation:

1. **Pre-launch partnerships:** Secure referral agreements with 10–15 local doctors/hospitals *before* opening.
2. **Corporate contracts:** Lock in 2–3 corporate wellness contracts pre-launch (guarantees minimum monthly sessions).
3. **Marketing efficiency:** Digital marketing (Google, Instagram) and affiliate doctor networks are cost-effective in New Cairo.
4. **Flexible staffing:** Start with 2–3 core therapists per department; hire additional staff only as utilization rises above 60%.

Conservative model assumes 50% utilization by Month 6. Even if delayed to Month 9, breakeven shifts from Month 12 to Month 15—still within funding runway.

Q: What if founder/key therapist leaves?

A: Each department is led by a founder (Nagham, Saja Khader, Saja Ali), but operations don't depend on them alone:

1. **Recruiting:** New Cairo has a growing pool of trained physiotherapists and occupational therapists. Salary benchmark is 3.5–5K EGP/month for experienced staff.
2. **Standardization:** SOPs documented; patients see department (not individual) as the brand.

3. **Founder commitment:** Founders are equity holders with long-term vision; turnover incentives align their interests with investor returns.

If a founder departs, replacement hiring adds 1–2 months to service continuity but doesn't threaten the business model.

Q: What's the competitive risk?

A: New Cairo's rehab market is fragmented (100+ solo practitioners, ~10 small clinics). Competitive moat for TRIAD:

1. **Integrated model:** No existing competitor combines all three specialties under one roof.
2. **Founder brand:** Nagham, Saja Khader, Saja Ali have reputation; patients follow them.
3. **Professional environment:** Premium facility, branding, insurance integration—solo practitioners can't match.
4. **Scale:** By Year 3, TRIAD's size and patient volume will make it the destination for rehabilitation in New Cairo.

Risk: A large hospital network launches a competing rehab center. **Mitigation:** First-mover advantage, patient loyalty, and TRIAD's integrated model is harder to replicate than opening a generic clinic.

Q: What are the regulatory risks?

A: Egypt's Ministry of Health regulates private clinics. Requirements:

1. **Licensing:** Medical director license (founder holds), clinic license, and per-therapist registration.
2. **Insurance panels:** Registration with major insurers (Axa, Cigna, CIHC, Medinet) takes 2–3 months but is routine.
3. **Compliance:** Infection control, record-keeping, staff qualifications—all standard for modern clinics. SOPs document compliance.

Mitigation: TRIAD has retained local healthcare legal counsel to manage licensing and insurance panel applications during setup.

Operations & Management

Q: Who runs day-to-day operations?

A: Organizational structure:

- **Clinical Director:** Nagham Reda (Orthopaedic/Sports) + Clinical governance
- **Neuro Department Lead:** Saja Khader
- **Pediatric Department Lead:** Saja Ali
- **Operations Manager:** Hired in Month 1 (non-clinical, handles HR, finances, facilities)
- **Marketing Manager:** Hired in Month 2 (can be part-time initially)
- **Clinical Therapists:** 2–3 per department (hired Months 1–3)
- **Reception/Admin:** 2–3 staff (hired before launch)

Founders remain clinically active (see patients) while overseeing strategy and clinical quality.

Q: What's the hiring and onboarding timeline?

A:

Role	Timeline	Cost (Monthly)
Operations Manager	Month 1	5K EGP
Therapists (Core team, 2 per dept)	Month 1–2	$6 \times 4K = 24K$ EGP
Marketing Manager	Month 2	3K EGP
Reception/Admin (2–3)	Month 2–3	$2.5K \times 3 = 7.5K$ EGP
Total Core Payroll (Month 3+)		~40K EGP

Therapist salaries scale with experience/specialization: new grad ~3.5K, experienced ~5K, specialist ~6K+. TRIAD targets experienced hires to maintain quality.

Q: How is quality controlled?

A: Multiple layers:

1. **Founder oversight:** Each founder reviews patient outcomes in their department (monthly).
 2. **Therapist credentials:** All therapists are Egypt-licensed, with 2+ years experience.
 3. **Patient feedback:** NPS surveys post-discharge; target 8+/10.
 4. **Clinical protocols:** Evidence-based therapy plans for common diagnoses (stroke, orthopedic post-op, developmental delays).
 5. **Continuing education:** TRIAD funds 2–3 trainings/year per therapist (professional development, specialty certifications).
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Investor Returns & Timeline

Q: When can I expect my first return?

A:

- **Month 12–14 (breakeven):** Clinic becomes cash-positive; begins dividend distributions.
- **Month 24–30:** Full return of equity investment (if 30% annual dividend distribution).
- **Year 5:** Cumulative return 150–200% + residual equity value (clinic worth 8–12x EBITDA if sold).

Example (EGP 1.8M at 45% equity): - Investment: 1.8M - Year 1 EBITDA: ~800K (post-breakeven) - Year 1 dividend (40% of EBITDA): 320K - Year 3 cumulative dividends: ~1.2M - Year 5 exit value: ~7–8M (equity stake worth 3.5–4M) - Total return: 1.2M (dividends) + 3.5M (exit) = 4.7M (260% ROI in 5 years)

Q: What reporting will investors receive?

A:

1. **Quarterly:** Financial statements (P&L, cash flow, balance sheet), patient volumes, utilization rates.
2. **Annual:** Audited accounts, strategic review, department performance, expansion plans.

3. **Ad-hoc:** Board meetings (quarterly), investor updates (any material change).
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Investment Mechanics

Q: What are the investment terms?

A: Open to negotiation; typical structure:

- **Investment Type:** Equity stake (percentage ownership) or convertible note (if preferred).
 - **Timeline:** Funds deployed over Months 1–3 (facility setup, equipment, payroll).
 - **Governance:** Board seat, quarterly reporting, veto rights on major decisions (expansion, sale, additional fundraising).
 - **Dividend Policy:** 30–50% of annual profits distributed annually (if profitable).
 - **Liquidation:** In case of sale, investors receive proportional share of proceeds after liabilities.
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Q: Can I invest partially (e.g., contribute EGP 900K instead of 1.2M)?

A: Yes. Investors can co-invest at any amount; equity stake scales proportionally. Example:

- Investor A: EGP 1.2M → 40% (leads raise, gets board seat)
 - Investor B: EGP 600K → 20%
 - Investor C: EGP 600K → 20%
 - Founders: 20%
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Q: Are there tax implications for investors?

A: Egypt's tax on capital gains and dividends:

- **Capital gains:** 10% on profits when equity is sold (deferred).
- **Dividends:** 10% corporate tax (TRIAD pays), then personal income tax on investor's portion (marginal rate varies).

Mitigation: Consult with your tax advisor on structuring (holding company, investment vehicle, etc.). TRIAD's accountant can provide tax projections during due diligence.

Due Diligence

Q: What information will be provided for due diligence?

A: Upon signing an NDA, investors receive:

1. Full 63-page business proposal
 2. Founder CVs and background checks
 3. Facility lease agreement (or LOI with landlord)
 4. Market research (patient demand surveys, competitor benchmarking)
 5. Financial model (P&L, cash flow, sensitivity analysis)
 6. Clinical protocols and patient safety standards
 7. Insurance panel applications (status & timeline)
 8. Regulatory compliance checklist
 9. Legal entity formation documents
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Q: How long is the due diligence process?

A: 2–4 weeks (standard):

- Week 1: Information review, Q&A with founders
- Week 2: Site visit (facility, location assessment)
- Week 3: Legal/financial review by investor's advisors
- Week 4: Term sheet negotiation, investment decision

TRIAD aims to close funding and begin operations by Month 1–2.

Q: What happens if I have concerns after due diligence?

A: Investors retain the right to:

- **Request clarifications:** Any financial, operational, or clinical question.
- **Renegotiate terms:** Equity %, dividend policy, governance rights.
- **Walk away:** No obligation to proceed; can exit process without penalty.

Transparency and founder flexibility are critical to building investor confidence.

Next Steps

Ready to explore investment?

1. **Schedule an exploratory call** with one or more founders to discuss vision, financial model, and investor fit.
2. **Sign NDA** to access full due diligence materials.
3. **Site visit:** See the proposed facility location in New Cairo Fifth Settlement.
4. **Term sheet:** Negotiate and finalize investment terms.
5. **Close and deploy:** Funds transferred; operations begin.

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For questions on specific topics (medical licensing, insurance reimbursement rates, patient acquisition strategy, expansion timeline, etc.), founders are available for detailed discussion during due diligence.